

SUBJECTIVE:

- Chief Complaint: Patient reports feeling unwell with severe, constant headache since yesterday morning.
- Onset: Headache started yesterday morning.
- Severity: Described as "very severe."
- Character: Headache is constant.
- Associated Symptoms: Denies nausea or vomiting. Patient briefly mentioned discomfort in the throat, but confirmed headache as the primary and only significant complaint. Denies fever, vision changes, weakness, numbness, or other neurological symptoms.
- Relieving Factors: None reported.
- Aggravating Factors: None specifically reported.

OBJECTIVE:

- General Appearance: Patient appears uncomfortable and distressed, holding their head.
- HEENT: Head normocephalic, atraumatic. Mild tenderness to palpation in bilateral temporal and frontal regions. Eyes: Pupils equal, round, reactive to light, no nystagmus. Fundoscopic exam not performed. Oropharynx: Mild erythema noted on posterior pharynx, no exudates or tonsillar hypertrophy. Neck: Supple, full range of motion, no meningeal signs (e.g., nuchal rigidity).
- Neurological: Alert and oriented to person, place, and time. Cranial nerves II-XII grossly intact. Motor and sensory systems intact bilaterally. Deep tendon reflexes symmetrical. No focal neurological deficits appreciated.
- Vital Signs: Not available at this time.

ASSESSMENT:

- Primary Diagnosis: Migraine without Aura. The severe, constant nature and lack of specific triggers or other symptoms point towards a primary headache disorder.
- Differential Diagnosis:
 - Tension-Type Headache: Less likely given the reported "very severe" intensity, but remains a possibility.
 - Sinusitis: Possible given the frontal tenderness and mild throat erythema, but typical purulent discharge or fever are absent.
 - Early Viral Illness: Vague "not feeling well" and mild throat discomfort could be prodromal for a common cold or other viral infection.
 - Eyestrain: Could contribute to headache, especially if prolonged screen time or uncorrected vision.

PLAN:

- Tests: No immediate laboratory or imaging tests indicated for this presentation. If symptoms persist or worsen, consider complete blood count (CBC), erythrocyte sedimentation rate (ESR), or cranial imaging (CT scan of head).
- Medications:
 - Over-the-counter analgesics: Advise Ibuprofen 400-600mg every 6-8 hours as needed, or Acetaminophen 500-1000mg every 6-8 hours as needed.
 - Consider prescription for a Triptan (e.g., Sumatriptan 50mg) if OTC options are ineffective and migraine diagnosis is strongly suspected, with instructions for early use at headache onset.
- Patient Advice:
 - Rest in a dark, quiet room.
 - Maintain adequate hydration.
 - Avoid known headache triggers such as stress, lack of sleep, or certain foods.
 - Apply a cold compress to the forehead.
- Follow-up Instructions: Return to clinic if headache worsens, new symptoms develop (e.g., fever, vision changes, weakness, severe neck stiffness), or if there is no significant improvement in symptoms within 24-48 hours.